QR Code for
report verification

Case ID	:	**2600132470**	**Sample Type**	:	**EDTA PERIPHERAL BLOOD**
Name	:	**MR. BIRENDRA KR TRIPATHI (KOL2604200324)**	**Date & Time Collected**	:	**20-Apr-2026 06:44 AM**
Sex/Age	:	**Male/49 Years**	**Date & Time Received**	:	**22-Apr-2026 01:03 PM**
Bill. Loc.	:	**OncQuest Laboratories Ltd., New Delhi**	**Date & Time Reported**	:	**20-May-2026 06:11 AM**
Ref. By	:		**Report Version**	:	**1**
Indication	:				

NGS Hereditary Cancer Panel
on the Illumina Novaseq X NGS Platform

Clinical Indication:

Birendra Tripathi diagnosed case of adenocarcinoma of ascending colon with brain lesion.

Sample Description:

Sample quality is optimum for the test.

Pathogenic deletion identified in relation to phenotype

Key Findings:

Gene & Transcript	Location	Variant	Zygosity/ Inheritance	Phenotype	Clinical Significance
MSH2 (+) NM_000251.3	2p21 (?_47,410,089-47,416,441_?) Exon 3-6	c.367-?_c.1076+? 6.4kb	Heterozygous /Autosomal Dominant	Lynch syndrome 1	Pathogenic

**Genetic test results are reported based on the recommendations of American College of Medical Genetics*

Variant Interpretation & Clinical correlation:

***MSH2*:Deletion Exon 3-6;**

The submitted sample shows Heterozygous deletion of exon 3-6 of gene *MSH2* on chromosome 2. A 6.4kb deletion detected on cytoband 2p21 (?_47,410,089-47,416,441_?). This variant is absent in our internal database.

Based on above evidence, this variant (MSH2:Deletion Exon 3-6) is classified as* Pathogenic *variant.

***MSH2 Gene Phenotype Description*:**

Lynch syndrome-1 (LYNCH1), also known as hereditary nonpolyposis colorectal cancer (HNPCC1), is caused by heterozygous mutation in the mismatch repair gene MSH2 (609309) on chromosome 2p21-p16. Lynch syndrome (LYNCH), also called hereditary nonpolyposis colorectal cancer, is defined as an autosomal dominant predisposition to a spectrum of cancers, primarily of the colorectum and endometrium, which exhibit impaired DNA mismatch repair (MMR) activity (summary by Lynch et al., 2015).

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Additional Findings:

Gene & Transcript	Location	Variant & Depth/VAF	Zygosity/ Inheritance	Phenotype	Clinical Significance
HBB (-) NM_000518.5 (chr11:g.5227071T>G)	Exon 1	c.-50A>C (5' prime UTR variant) 49x/51%	Heterozygous /Autosomal Recessive	Beta-thalassemia HBB	Pathogenic (PM2, PM3, PP5)

**Genetic test results are reported based on the recommendations of American College of Medical Genetics*

HBB Gene Phenotype Description:

Direct sequencing of specific regions of genomic DNA became feasible with the invention of the polymerase chain reaction (PCR) which permits amplification of specific regions of DNA. Recently, human mitochondrial DNA was amplified and directly sequenced. Using a thermostable DNA polymerase of T. aquaticus (Saiki, R.K. et al., manuscript in preparation) in the PCR, we have applied a combination of PCR and direct sequence analysis of the amplified product to a human single-copy gene. We studied the genomic DNA of five patients with beta-thalassaemia whose mutant alleles were uncharacterized, and found two previously undescribed mutations, along with three known alleles. One new allele is a frameshift at codons 106-107 and the other is an A-C transversion at the cap site (+1) of the beta-globin gene. This latter is the first natural mutation observed at the cap site and it occurs in a gene which is poorly expressed.

Case Specific Recommendations:

1. Segregation analysis of the variant by Sanger /Exome sequencing/ MLPA is recommended.

Recommendations

- Genetic counseling is advised for interpretation on the consequences of the variant(s).
- We recommend confirming the presence of these variants by Sanger Sequencing/ MLPA.
- Segregation analysis of the variant by Sanger /Exome sequencing/ MLPA is recommended.
- The significance/classification of the variants might change based on parental and family members genetic testing.
- For questions about this report, or for assistance in locating nearby genetic counseling services, please contact the Laboratory: contact@sngenelab.com.
- If results obtained do not match the clinical findings, additional testing should be considered as per referring clinician's recommendation.

Methodology:

- Next Generation Sequencing: DNA extracted from Blood, Saliva, Amniotic fluid, CVS or any other standard source is used for targeted capture-based Library preparation. Targeted capture provides an efficient and sensitive means for sequencing specific genomic regions in a high-throughput manner. The libraries were sequenced to mean >85-100x coverage on Illumina Novaseq X sequencing platform with Paired End 2x150 chemistry.
- We follow the Illumina DRAGEN Bio-IT Platform for identification of variants in the sample. The sequences obtained are assembled and aligned to reference sequences based on NCBI RefSeq transcripts and human genome build (GRCh38).

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- This assay (Whole exome sequencing) covers 20321 genes including 37 mitochondria genes.
- Haplotype caller has been used to identify variants which are relevant to the clinical indication.
- In addition to SNVs and small Indels, copy number variants (CNVs) are detected from targeted sequence data using the commercially available algorithm. This algorithm detects rare CNVs based on comparison of the read-depths of the test data with the matched aggregate reference dataset.
- Clinically relevant mutations were annotated using published variants in literature, Commercial datasets and a set of diseases databases. Common variants are filtered based on allele frequency in 1000Genome, ExAC, gnomAD, dbSNP & SN Genelab's internal database.
- Non-synonymous variants effect is calculated using multiple algorithms such as PolyPhen-2, SIFT, MutationTaster2.
- Based on annotations data, ACMG rules-based classification performed for classification of variants identified through next generation sequencing study.

Limitations/Disclaimer:

- It should be noted that this test does not sequence all bases in a human genome, not all variants have been identified or interpreted, and this report is limited only to variants with evidence for causing or contributing to disease/clinical details provided to SN Gene Lab Pvt Ltd.
- Testing has been performed assuming that the sample received belongs to the above named individual and any stated relationships between individuals are accepted as true.
- Test results are interpreted in the context of clinical findings, family history and other laboratory data. Only variations in genes potentially related to the proband's medical condition are reported. Rare polymorphisms may lead to false negative or positive results. Misinterpretation of results may occur if the information provided is inaccurate or incomplete. The results should be interpreted in the context of the patient's medical evaluation, family history and racial/ethnic background. Please note that variant classification and/or interpretation may change over time if more information available. Re-interpretation of multi gene next generation sequencing data is recommended on an annual basis and may be requested by a medical provider.
- The sensitivity of this assay to detect large deletions/duplications of more than 10 bp or copy number variations (CNV) is 70-75%. The CNVs detected with this assay have to be confirmed by alternate method such as MLPA & Microarray.
- Due to inherent technology limitations, coverage is not uniform across all regions. Hence pathogenic variants present in areas of insufficient coverage as well as those variants which currently do not co-relate with the provided phenotype may not be analyzed/ reported. Additionally, it may not be possible to fully resolve certain details about variants, such as mosaicism, phasing, or mapping ambiguity.
- Triplet repeat expansions, translocations, large deletion or duplications and chromosomal rearrangements events are currently not reliably detected by next generation sequencing.
- This assay is not meant to interrogate most promoter regions, deep intronic regions, or other regulatory elements, other gene rearrangements like inversion or translocation and does not detect single or multi-exon deletions or duplications.
- Incidental or secondary findings (if any) that meet the ACMG guidelines can be given upon request.
- Genes with pseudogenes, paralog genes and genes with low complexity may have decreased sensitivity and specificity of variant detection and interpretation due to inability of the data and analysis tools to unambiguously determine the

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origin of the sequence data in such regions.

- Sequence and copy number variants are reported according to the Human Genome Variation Society (HGVS).
- The transcript used for clinical reporting generally represents the canonical transcript, which is usually the longest coding transcript with strong/multiple supporting evidence. However, clinically relevant variants annotated in alternate complete coding transcripts could also be reported.

*Variant classification as per ACMG guidelines:	
Variant	A change in a gene. This could be disease causing (pathogenic) or not disease causing (benign).
Benign	A variant which is known not to be responsible for disease has been detected. Generally, no further action is warranted on such variants when detected.
Likely Benign	A variant which is very unlikely to contribute to the development of disease however, the scientific evidence is currently insufficient to prove this conclusively. Additional evidence is expected to confirm this assertion of Pathogenicity.
Pathogenic	A disease-causing variation in a gene which can explain the patient's symptoms has been detected. This usually means that a suspected disorder for which testing had been requested has been confirmed.
Likely Pathogenic	A variant which is very likely to contribute to the development of disease however, the scientific evidence is currently insufficient to prove this conclusively. Additional evidence is expected to confirm this assertion of pathogenicity.
Variant of Uncertain Significance	A variant has been detected, but it is difficult to classify it as either pathogenic (disease causing) or benign (non-disease causing) based on current available scientific evidence. Further testing of the patient or family members as recommended by your clinician may be needed. It is probable that their significance can be assessed only with time, subject to availability of scientific evidence.

References:

1. Richards S, Aziz N, Bale S, et al. Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology, Genetics in Medicine, 2015 May;17(5):405-24

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5. 1000 Genomes Project Consortium et al., A global reference for human genetic variation. Nature, 526(7571): 68-74, 2015.

6. Lek M. et al., Analysis of protein-coding genetic variation in 60,706 humans. Nature, 536(7616):285-91, 2016.

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Appendix 1: SAMPLE DATA AND STATISTICS

Title	Data
Total data generated (Gb)	8.09
Total Reads (%)	100.00
Total reads aligned (%)	99.30
Data >Q30 (%)	96.93

Appendix 2: COVERAGE SUMMARY

Mean Depth	Percentage target base pairs covered		
	1x	5x	20x
80.16x	98.71%	98.42%	97.85%

Appendix 3: MTDNA GENES

Gene	Region covered	*Gene*	Region covered	Gene	Region covered
MT-ATP6	100.00%	*MT-ND6*	100.00%	*MT-TL1*	100.00%
MT-ATP8	100.00%	*MT-RNR1*	100.00%	*MT-TL2*	100.00%
MT-CO1	100.00%	*MT-RNR2*	100.00%	*MT-TM*	100.00%
MT-CO2	100.00%	*MT-TA*	100.00%	*MT-TN*	100.00%
MT-CO3	100.00%	*MT-TC*	100.00%	*MT-TP*	100.00%
MT-CYB	100.00%	*MT-TD*	100.00%	*MT-TQ*	100.00%
MT-ND1	100.00%	*MT-TE*	100.00%	*MT-TR*	100.00%
MT-ND2	100.00%	*MT-TF*	100.00%	*MT-TS1*	100.00%
MT-ND3	100.00%	*MT-TG*	100.00%	*MT-TS2*	100.00%
MT-ND4	100.00%	*MT-TH*	100.00%	*MT-TT*	100.00%
MT-ND4L	100.00%	*MT-TI*	100.00%	*MT-TV*	100.00%
MT-ND5	100.00%	*MT-TK*	100.00%	*MT-TW*	100.00%
MT-TY	100.00%				

Appendix 4: COVERAGE OF ANALYZED GENES (Percentage of coding region covered)

Gene	Region covered	Gene	Region covered	Gene	Region covered
RAD50	100.00%	*CDK4*	100.00%	*CDKN1B*	100.00%
CDKN2A	100.00%	*CHEK2*	100.00%	*FLCN*	100.00%
FH	100.00%	*MSH6*	100.00%	*APC*	100.00%
EPCAM	100.00%	*SMAD4*	100.00%	*MAX*	100.00%
MEN1	100.00%	*MET*	100.00%	*MITF*	100.00%
MLH1	100.00%	*MRE11*	100.00%	*MSH2*	100.00%
MUTYH	100.00%	*NBN*	100.00%	*ATM*	100.00%
NF1	100.00%	*PMS2*	100.00%	*SDHAF2*	100.00%
TMEM127	100.00%	*PRKAR1A*	100.00%	*PTCH1*	100.00%
PTEN	100.00%	*BARD1*	100.00%	*RAD51C*	100.00%
RAD51D	100.00%	*RB1*	100.00%	*RET*	100.00%
SDHB	100.00%	*BLM*	100.00%	*BMPR1A*	100.00%
BRCA1	100.00%	*BRCA2*	100.00%	*STK11*	100.00%
TP53	100.00%	*TSC1*	100.00%	*TSC2*	100.00%
VHL	100.00%	*XRCC2*	100.00%	*CDC73*	100.00%
PALB2	100.00%	*BAP1*	100.00%	*BRIP1*	100.00%
CDH1	100.00%	*AXIN2*	100.00%	*CTNNA1*	100.00%
DICER1	100.00%	*GREM1*	100.00%	*HOXB13*	100.00%
KIT	100.00%	*MSH3*	100.00%	*NTHL1*	100.00%
PDGFRA	100.00%	*POLD1*	100.00%	*POLE*	100.00%
SDHA	100.00%	*SDHC*	100.00%	*SDHD*	100.00%
SMARCA4	100.00%	*VPS13B*	100.00%	*RNASEL*	100.00%
ELAC2	100.00%	*MSR1*	100.00%	*PRSS1*	100.00%
SPINK1	100.00%	*ABRAXAS1*	100.00%	*ACVRL1*	100.00%
AKT1	100.00%	*BUB1B*	100.00%	*CASR*	100.00%
CDKN1C	100.00%	*CEBPA*	100.00%	*DDX41*	100.00%
DIS3L2	100.00%	*EGFR*	100.00%	*ETV6*	100.00%
EXT1	100.00%	*EXT2*	100.00%	*FANCC*	100.00%
GALNT12	100.00%	*GATA2*	100.00%	*GPC3*	100.00%
HNF1A	100.00%	*HNF1B*	100.00%	*HRAS*	100.00%
KIF1B	100.00%	*MC1R*	100.00%	*MLH3*	100.00%
NF2	100.00%	*PHOX2B*	100.00%	*PIK3CA*	100.00%
PMS1	100.00%	*POT1*	100.00%	*PTCH2*	100.00%
RECQL	100.00%	*REST*	100.00%	*RNF43*	100.00%
RPS20	100.00%	*RUNX1*	100.00%	*SAMD9L*	100.00%
SMARCA2	100.00%	*SMARCB1*	100.00%	*SMARCE1*	100.00%
SUFU	100.00%	*TERT*	100.00%	*TGFBR2*	100.00%

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TRIP13	100.00%	*WRN*	100.00%	*WT1*	100.00%
XRCC3	100.00%	*CDKN2B*	100.00%	*AIP*	100.00%
ALK	100.00%	*ANKRD26*	100.00%	*BRAF*	100.00%
CBL	100.00%	*CD70*	100.00%	*CEP57*	100.00%
CYLD	100.00%	*DDB2*	100.00%	*DKC1*	100.00%
EFL1	100.00%	*ELANE*	100.00%	*ERCC1*	100.00%
ERCC2	100.00%	*ERCC3*	100.00%	*ERCC4*	100.00%
ERCC5	100.00%	*EXO1*	100.00%	*EZH2*	100.00%
FAM111B	100.00%	*FANCA*	100.00%	*FANCB*	100.00%
FANCD2	100.00%	*FANCE*	100.00%	*FANCF*	100.00%
FANCG	100.00%	*FANCI*	100.00%	*FANCL*	100.00%
FANCM	100.00%	*GPR101*	100.00%	*HAVCR2*	100.00%
IKZF1	100.00%	*KITLG*	100.00%	*KRAS*	100.00%
LZTR1	100.00%	*MAP2K1*	100.00%	*MAP2K2*	100.00%
NRAS	100.00%	*NSD1*	100.00%	*NSUN2*	100.00%
PAX5	100.00%	*POLH*	100.00%	*PPM1D*	100.00%
PRF1	100.00%	*PTPN11*	100.00%	*RAF1*	100.00%
RASA2	100.00%	*RECQL4*	100.00%	*RHBDF2*	100.00%
RIT1	100.00%	*RRAS*	100.00%	*SAMD9*	100.00%
SBDS	100.00%	*SHOC2*	100.00%	*SLX4*	100.00%
SOS1	100.00%	*SOS2*	100.00%	*SPRED1*	100.00%
SRP72	100.00%	*TINF2*	100.00%	*XPA*	100.00%
XPC	100.00%	*CPA1*	100.00%	*RFWD3*	100.00%
BTNL2	100.00%	*NHEJ1*	100.00%	*RINT1*	100.00%
AR	100.00%	*CD82*	100.00%	*ATR*	100.00%
PALLD	100.00%	*ZFHX3*	100.00%	*AXIN1*	100.00%
TGFB1	100.00%	*ENG*	100.00%	*RAD51*	100.00%
LIG4	100.00%	*TOX3*	100.00%	*LSP1*	100.00%
RASAL1	100.00%	*MXI1*	100.00%	*CFTR*	100.00%
FGFR2	100.00%	*MAD2L2*	100.00%	*MYSM1*	100.00%
MAP3K1	100.00%	*UBE2T*	100.00%	*ACD*	100.00%
KLHDC8B	100.00%	*NPAT*	100.00%	*TERF2IP*	100.00%
ADA	100.00%	*CARD11*	100.00%	*CASP10*	100.00%
CD27	100.00%	*CTLA4*	100.00%	*DOCK8*	100.00%
FAS	100.00%	*FASLG*	100.00%	*ITK*	100.00%
MAGT1	100.00%	*PIK3CD*	100.00%	*SH2D1A*	100.00%
APCS	100.00%	*STXBP2*	100.00%	*TNFRSF13B*	100.00%
WAS	100.00%	*CARMIL2*	100.00%	*CASP8*	100.00%
CTPS1	100.00%	*FADD*	100.00%	*FCHO1*	100.00%
IL2RA	100.00%	*IL2RB*	100.00%	*MCM4*	100.00%

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PIK3R1	100.00%	*PRKCD*	100.00%	*RAC2*	100.00%
RASGRP1	100.00%	*RHOH*	100.00%	*STAT3*	100.00%
STK4	100.00%	*TPP2*	100.00%	*XIAP*	100.00%
CREBBP	100.00%	*MYD88*	100.00%	*STAT6*	100.00%
B2M	100.00%	*EP300*	100.00%	*NOTCH1*	100.00%
TET2	100.00%	*BCL2*	100.00%	*NOTCH2*	100.00%
TNFAIP3	100.00%	*FOXO1*	100.00%	*TNFRSF14*	100.00%
GNA13	100.00%	*PIM1*	100.00%	*CD79A*	100.00%
KMT2D	100.00%	*CD79B*	100.00%	*MYC*	100.00%
SOCS1	100.00%	*FAN1*	100.00%	*CDK12*	100.00%
CHEK1	100.00%	*CUL2*	100.00%	*DIS3*	100.00%
ERBB2	100.00%	*ESR2*	100.00%	*TDO2*	100.00%
GFAP	100.00%	*MED12*	100.00%	*PPP2R2A*	100.00%
RAD54L	100.00%	*SLC45A2*	100.00%	*SPOP*	100.00%
TYR	100.00%	*MBD4*	100.00%	*HBB*	100.00%

------------------ End Of Report ------------------

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